

VISIT · VICTR, VANDERBILT UNIVERSITY MEDICAL CENTER · 8 OCTOBER 2026

Valuing the Hub Function

Crediting support for other institutions' research



Toshiki Saito, MD, PhD

Professor / Director, Clinical Research Promotion Center
The University of Tokyo Hospital

VICTR, Nashville

8 October 2026

Where we stand

It shapes what we hope to learn.

Hospital

The University of Tokyo Hospital is one of Japan's 16 Core Clinical Research Hospitals.
The criteria I describe today apply to the center I run.

National

Representative Secretary, NUH-CRPI
The council of all national university hospitals: 42 universities, 44 hospitals.

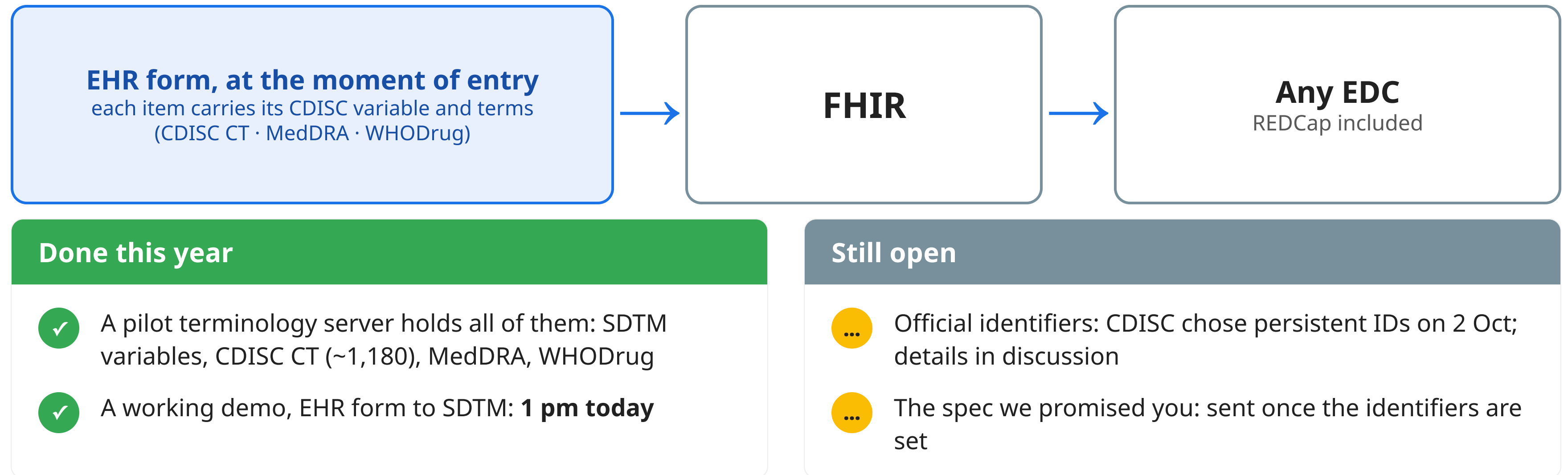
Data

Vice Chair, Japan CDISC Coordinating Committee · AMED principal investigator
Clinical data standards. This layer comes next.

Not one hospital's site visit: we came for a national system that is deciding how to measure its research hubs.

A year on from our last visit

You asked whether our EDC would connect to the EHR. Our answer starts one step earlier.



Japan's Core Clinical Research Hospitals

Created by law in 2015. The Minister approves hospitals that meet published criteria; 16 hold the designation today.

CTSA award (US)

Nature	A grant
Entry	Compete and win
Review	Periodic; the award can end
Steering	A national program office

Core hospital (Japan)

Nature	A legal status
Entry	Meet the published criteria
Review	Kept until it fails the criteria
Steering	No program office, no peer competition

The criteria are the whole instrument, so what they count matters.

What the criteria count

Three things, measured over the previous three years.

8

Trials

Investigator-initiated trials for drug approval, or 4 of them plus 40 other trials

In principle, led by the hospital's own investigator

45

Papers

Peer-reviewed English papers

In principle, the first author is from the hospital

15

Supports

Studies supported at other institutions, per year. Mandatory, but a floor

15 and 50 score the same

A study led from outside counts only if the hospital's ARO takes on **the whole package**: protocol, data management and monitoring.

The hub function is what the instrument measures least.

What happens as a result

What we see from inside the Core Hospitals: our observation, not yet a quantified study.

- ✕ **Small, easy-to-start trials.** Eight in three years is a hard number, and a small trial counts the same as a large one.
- ✕ **More papers.** Forty-five is a hard number too.
- ✕ **Support is penalized.** Every statistician, data manager and dollar moved to another institution's study is not spent on the hospital's own count.

The Ministry knows this. Its review this year keeps support as a floor, because the effort behind support varies too much to be scored.

That is exactly the problem your funders have had to solve.

Three shifts we propose

Re-weight counting, do not abolish it. Each shift has a working counterpart in how NIH funds research infrastructure.

1 Input → outcome

Count trials that finish and report. Judge approvals and guideline changes case by case, over a longer period.

2 In-house → support

Credit research the hub enabled at other institutions as the hub's own achievement.

3 Single site → network

Cross-cutting infrastructure at the core; disease-specific networks carry the rest.

NIH review, in the funding announcements we read (CTSA hubs, TIN, coordinating centers): no in-house trial counts, no first-author papers, no points or weights. Reviewers judge coordination, data and statistics, consortium contribution and dissemination. **Colorado confirmed it yesterday:** the annual report is read, not scored, and a paper counts whoever its author is.

Two layers of NIH money, FY2025

Base layer: CTSA hubs

\$7–11M (¥1.1–1.8B) per hub, set by a tier on NIH funding, not on performance

\$0.57B (¥91B)

Competitive layer

coordinating, data and statistical centers

\$1.0B (¥160B) · where institutions differ · what Japan lacks

What we came to ask

Eight written questions: your agenda gives each one a session today.

8:10 Community engagement Q8	8:40 Coordinating Center Sean Collins Q3	10:00 Metrics, money and shared infrastructure Paul Harris Q1Q2Q4Q6	11:00 Research support workforce Terri Edwards Q5	11:30 EHR data and BioVU Q7
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So let me go straight to what we most want to learn.

What we most want to learn

Three questions for this morning with Paul.

1

The review itself

What decided your last renewal? Colorado told us the annual report is read but never scored, and a score comes only at renewal. Is that your experience too?

2

Our question six

Colorado said an in-house minimum would change nothing for them: 90% of what they support is on their own campus. VICTR is the opposite case. Could you keep REDCap and the Trial Innovation Network open to others?

3

All of Us

Your largest federal award is a national data center awarded outside ordinary peer review. How is it run, and what is it judged on, year to year?



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Thank you



Now I would much rather hear from you.